

1 Interview Transcript with Dr. Anjali Singh (Lucknow, India) - Senior Consultant, Pediatrics and 2 Neonatology, City Children's Hospital

3 Interviewer: Dr. Singh, thank you for joining us. To start, could you tell us about your
4 background and how your role changed during the pandemic? Dr. Singh: Thank you. I'm Dr. Anjali
5 Singh, a pediatrician and neonatologist practicing in Lucknow for the past 17 years. My work
6 involves managing newborns, infants, and children in both outpatient and hospital settings.
Before COVID-19, pediatrics was about growth milestones, immunization, and infections like
pneumonia or dengue.

When COVID arrived, everything changed. Initially, we thought children were largely spared. But
as the waves progressed, we began seeing severe cases, especially during Delta. Beyond direct
infections, the pandemic disrupted immunizations, nutrition programs, and mental health among
children. My role evolved from clinician to counselor, educator, and sometimes even crisis
manager for families navigating fear and misinformation.

Interviewer: What were your first experiences treating children with COVID-19 like? Dr. Singh:
The first wave in 2020 brought mostly mild or asymptomatic cases in children. We saw them
mainly as contacts of infected parents. But the second wave was different - more children with
high fever, dehydration, and in rare cases, respiratory distress.

One memorable case was a 5-year-old with COVID-induced pneumonia. The challenge was
balancing
oxygen therapy with the fear in the child's eyes. Pediatric care is about trust; masks and PPE
made that harder. Children couldn't see our faces. We began wearing cartoon stickers on our PPE
suits to make them less intimidating. Small things like that mattered more than any medicine.

Interviewer: What about the indirect effects on children's health during lockdowns? Dr. Singh:
That's where the real damage occurred. For months, routine immunizations were delayed,
especially in rural and peri-urban areas. Nutritional programs like the mid-day meal scheme
were suspended, worsening malnutrition. We saw children coming in with weight loss, anemia, and
developmental delays.

Mental health suffered too. Children missed school, friends, outdoor play. Anxiety,
irritability, and sleep issues became common. Teenagers struggled with loneliness and screen
addiction. Pediatricians became informal psychologists - listening, reassuring, and teaching
parents how to create structure at home.

Interviewer: How did your hospital manage pediatric COVID wards? Dr. Singh: We set up a
separate pediatric COVID unit during the Delta wave, with isolation rooms for mothers and
children together whenever possible. Separating them was traumatic, especially for infants. We
trained nurses to wear transparent face shields so babies could still see facial expressions.

One of the hardest aspects was PPE inside neonatal ICUs. Communicating with crying infants

while muffled behind layers of protection felt inhuman at times. Yet, our nurses were remarkable - singing lullabies from behind masks, patting incubators through gloves. Humanity found its way through barriers.

Interviewer: Did you encounter any severe or unusual complications among children? Dr. Singh: Yes, the MIS-C cases - Multi-System Inflammatory Syndrome in Children - were our biggest concern. It usually appeared weeks after infection, even in children who had mild or asymptomatic COVID. These children came in with persistent fever, rash, low blood pressure, and cardiac involvement. We treated several with IV immunoglobulin and steroids. Early recognition was key.

We also saw increased post-viral asthma and autoimmune flares. Thankfully, mortality among children remained low, but the psychological trauma for families was immense.

Interviewer: How did vaccination for adolescents change the landscape? Dr. Singh: It was a turning point. When India approved vaccines for 15 18-year-olds, we organized school-based vaccination drives in coordination with the district health department. Many children were excited - they'd seen what COVID had done to their families. Others were scared of needles or rumors.

We made the process child-friendly - colorful posters, music, doctors dressed in cartoon scrubs. I remember one 16-year-old telling me, "It feels like we're getting our freedom back." That moment symbolized hope - for them and for us.

Interviewer: You mentioned misinformation. What were some common myths you encountered among parents? Dr. Singh: There were many. Early on, rumors spread that children couldn't catch COVID, so parents ignored mild symptoms. Later, social media created panic - stories about vaccine infertility, growth problems, or government conspiracies. We had to spend as much time correcting misinformation as treating illness.

We organized community awareness camps, using pediatricians as trusted messengers. We spoke in local languages, through radio and WhatsApp groups, explaining simply: "The vaccine teaches your child's body to fight, not harm it." The key was empathy, not argument.

Interviewer: How did you handle emotional situations, especially when families were separated? Dr. Singh: That was the hardest part of my career. Imagine telling a mother she can't touch her sick child because she tested positive herself. We arranged video calls so parents could see their children daily. I remember a father who cried seeing his 3-year-old wave through a screen.

We also faced deaths - rare in pediatrics, but devastating when they happened. We held small remembrance ceremonies for each child, with their names written on a "Wall of Love" in the

ward. It helped staff and families process grief together.

Interviewer: How did you manage the psychological wellbeing of your own staff? Dr. Singh: Pediatric care is emotionally draining even in normal times, but during COVID it was relentless. We created a rotating shift system to avoid burnout and encouraged open debriefing. I told my residents, "It's okay to cry. It means you still care."

We also started weekly team check-ins - not medical briefings, just conversations about how everyone was coping. Our nurses were the backbone - many were young mothers themselves, yet they cared for others' children while being away from their own.

Interviewer: How did the pandemic affect pediatric practice as a discipline in India? Dr. Singh: It redefined our priorities. Pediatricians used to focus narrowly on diseases; now we talk about child resilience, public health education, and digital wellbeing. Telepediatrics took off - we consulted children from villages hundreds of kilometers away. That model is here to stay.

The pandemic also underscored the need for pediatric preparedness - child-sized masks, ventilators, and doses were initially lacking. We learned that emergency planning often forgets children, but they're not small adults - their needs are unique.

Interviewer: Were there any silver linings from your perspective? Dr. Singh: Yes. The solidarity among healthcare workers was incredible. Pediatricians from government, private, and charitable sectors came together to share data and protocols. Parents became more health-conscious and communicative. Children adapted faster than adults - they wore masks cheerfully and reminded their parents to sanitize!

Also, digital learning and telehealth opened new ways to reach rural families. For example, we started remote neonatal consults for smaller hospitals - a practice that continues even now. Crises accelerate change, and this one forced healthcare to modernize.

Interviewer: What lessons do you believe India should take forward for future child health crises? Dr. Singh: Three big ones: 1. Never suspend preventive care. Immunizations and nutrition programs must continue even during lockdowns. 2. Invest in child mental health. We need pediatric counselors in every district. 3. Empower pediatricians in public health planning. Our voice must be part of every preparedness discussion - from schools to hospitals. Children may not vote or speak loudly, but they represent the country's future. Their needs should be central, not peripheral.

Interviewer: Finally, what has this pandemic taught you personally? Dr. Singh: It reminded me that medicine is not just a science - it's a relationship. Healing children requires touch, laughter, and trust - things we lost behind masks. I learned to communicate with my eyes more than words.

I also learned resilience from my patients. Children who lost parents still smiled when given crayons. They taught me that recovery isn't just physical - it's emotional.

When the last COVID wave ended, we reopened our playroom in the pediatric ward. Watching children draw suns and rainbows felt symbolic - light after darkness.

Interviewer: That's beautiful, Dr. Singh. Thank you for your compassion and your service. Dr.

Singh: Thank you. The pandemic tested us, but it also reminded us of the heart of medicine - to care, no matter what.

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